



February 23, 2026

Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy
Office of the National Coordinator for Health Information Technology
Attention: HHS Health Sector AI RFI
330 C Street S.W.
Washington DC 20201

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Dr. Keane:

RadNet, Inc. appreciates the opportunity to comment on the Office of the National Coordinator for Health Information Technology (ONC) and the Assistant Secretary for Technology Policy's (ASTP) Request for Information on accelerating the adoption and use of artificial intelligence (AI) as part of clinical care.

Background and Experience

RadNet is the nation's largest provider of outpatient diagnostic imaging services, operating more than 400 imaging centers across multiple states, many of which serve rural and underserved communities. Through our DeepHealth division, RadNet is also a leader in the development and deployment of FDA-cleared, AI-enabled imaging technologies for breast, lung, prostate, and other cancers—conditions that collectively account for approximately 70 percent of cancers amenable to imaging-based screening.

AI-enabled imaging technologies already are improving patient care today. With the appropriate regulatory clarity, reimbursement alignment, and data infrastructure, AI can significantly improve diagnostic accuracy, reduce unnecessary downstream utilization, lower costs, and expand access to high-quality care. Case in point, optimizing AI technology is essential to bridge the gap between the demand for imaging and the supply of radiologists, especially those with the appropriate subspecialty training. Rather than pushing studies out further and further which delays care and contributes to burnout, the future potential of AI is to make the radiologists (and radiological technologists) we do have able to work faster, smarter, and more accurately. The earlier disease detection that flows from the above comments will benefit our healthcare system by becoming far more accessible and less expensive.



RadNet's Comments on Specific Issues and Questions from the RFI

I. Regulation

RadNet supports predictable, risk-based oversight without creating duplicative requirements.

AI-enabled clinical technologies—particularly AI incorporated into medical devices—are already subject to rigorous federal oversight. The Food and Drug Administration (FDA) has established a mature, risk-based regulatory framework for AI-enabled medical devices, including premarket review, post-market surveillance, and requirements related to safety, effectiveness, and performance across diverse populations. ONC and ASTP should avoid creating duplicative or conflicting regulatory regimes that would add uncertainty, slow innovation, or discourage provider adoption. Instead, HHS should:

- **Reinforce reliance on FDA's existing authority** for AI-enabled clinical tools that meet the definition of a medical device.
- **Avoid duplicative or overlapping regulatory regimes** that would create confusion or conflicting requirements for developers and providers.
- **Promote inter-agency alignment**, ensuring that ONC policies related to health IT, interoperability, and transparency complement—rather than complicate—FDA oversight.
- **Ensure regulatory requirements are proportionate to risk**, recognizing that most imaging AI tools function as clinical decision support intended to assist clinicians, not replace them.

Over-regulation or fragmented oversight risks delaying patient access to AI tools that are already demonstrating measurable improvements in diagnostic accuracy, efficiency, and outcomes—contrary to the goals articulated in this RFI.

II. Reimbursement

Payment policy should align with clinical value to drive adoption.

Reimbursement remains one of the most significant barriers to the adoption of clinically validated AI technologies. Developing, validating, and maintaining high-performing healthcare AI is resource-intensive. Absent clear and predictable reimbursement pathways, providers—particularly independent imaging centers and community-based practices—will be slow or unable to adopt these technologies, regardless of their clinical value.

A prime example, the Medicare Physician Fee Schedule (MPFS) is particularly challenging for AI innovators. CMS previously had considered most AI services to be a physician office “indirect practice expense,” akin to an overhead cost like a computer system or rent for office space. However, many, if not most, AI services are provided to patients on a per-patient, per-service basis, which is best accounted for by a per-use payment, the same way that other medical services are valued and paid. Payment for AI-enabled medical technology under the MPFS is particularly difficult because CMS does

not have a formal payment pathway to support the adoption of innovative and new services that utilize AI technology. This is in contrast to other Medicare payment systems in use for hospitals which provide additional payment for services that incorporate new technologies through the New Technology Add-On Payment (NTAP) and the New Technology Ambulatory Payment Classifications (APC), respectively. CMS has discussed the potential need for a comprehensive update to PE data inputs, and possibly the PE methodology as a whole. CMS has expressed its belief that comprehensive PE data inputs and a different PE calculation methodology would better account for the use of AI technology, among other innovations.

HHS should consider policies that:

- **Establish consistent and predictable Medicare payment pathways** for FDA-cleared AI-enabled clinical technologies.
- **Avoid one-size-fits-all reimbursement models**, recognizing that AI generates value differently depending on modality, clinical context, and patient population.
- **Encourage coverage policies that recognize downstream savings**, including reductions in unnecessary follow-up imaging, biopsies, delayed diagnoses, and avoidable treatment costs.

In imaging-based cancer screening, AI has demonstrated the ability to improve diagnostic accuracy while reducing false positives and unnecessary callbacks—benefits that accrue to patients, providers, and payers. Payment policy should reflect these system-level efficiencies, consistent with the objectives outlined in this RFI.

III. Research & Development

Enable Responsible Access to High-Quality Data.

Access to large, diverse, and high-quality datasets is essential to the development of safe, effective, and unbiased healthcare AI. Radiology has been a leader in this area due to widespread digitization, PACS adoption, and adherence to the DICOM standard. However, even in imaging, AI development would be significantly enhanced by improved access to relevant clinical context, such as pathology results, surgical history, and longitudinal outcomes.

RadNet recommends that HHS:

- **Promote interoperability and standardized data exchange across care settings**, enabling AI tools to incorporate relevant clinical context.
- **Balance privacy protections with innovation**, ensuring policies do not unnecessarily restrict the use of appropriately de-identified data for AI training, validation, and monitoring.
- **Encourage real-world performance evaluation**, allowing AI tools to be assessed and improved based on how they function in diverse clinical environments.

Conversely, policies that overly constrain access to anonymized or de-identified data risk entrenching bias, limiting generalizability, and slowing progress are inconsistent with the goals of this RFI.

IV. Specific Questions in the RFI

1. *What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?*

The most significant barriers to private sector innovation and adoption of AI in clinical care include:

- **Regulatory uncertainty and fragmentation**, particularly when multiple HHS agencies consider overlapping requirements for clinical AI already regulated by FDA.
- **Lack of predictable reimbursement pathways**, even for FDA-cleared AI tools with demonstrated clinical benefit.
- **Limited access to high-quality, representative clinical data**, particularly longitudinal outcomes and non-imaging clinical context.
- **Interoperability gaps** that prevent AI tools from integrating seamlessly into clinical workflows.
- **Administrative burden on providers**, including contracting, IT integration, and compliance obligations that disproportionately affect community-based providers.

Absent clarity and alignment across regulation, payment, and infrastructure, even high-performing AI tools face slow and uneven adoption.

2. *What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What regulations, policies, or programs should be revisited?*

HHS should prioritize policies that **reduce uncertainty, avoid duplication, and align incentives with clinical value**, including:

- **Regulatory Priorities**
 - **Deference to FDA oversight** for AI-enabled medical devices, including diagnostic imaging AI, which are already subject to risk-based review and post-market surveillance.
 - **Avoidance of duplicative ONC certification or disclosure requirements** that overlap with FDA authorities.
 - **Clear delineation of ONC's role** around interoperability, transparency, and data exchange—not clinical performance validation.
- **Payment Policy Priorities**

- **Creation of predictable Medicare reimbursement pathways** for FDA-cleared AI tools that demonstrably improve diagnostic accuracy, efficiency, or patient outcomes.
- **Recognition of downstream value**, including reduced false positives, fewer unnecessary follow-ups, and earlier disease detection.
- **Flexibility within CMS payment models** to recognize AI's role in improving care quality without forcing modality-specific or one-size-fits-all payment structures.

Without aligned payment policy, adoption will remain limited regardless of regulatory clearance.

3. *For non-medical devices, what novel legal and implementation issues exist and what role should HHS play?*

For non-medical device AI used in clinical care—such as workflow optimization, triage, scheduling, or operational decision support—novel issues include:

- **Unclear liability and accountability frameworks**, particularly when AI influences but does not determine clinical decisions.
- **Privacy and data security concerns**, especially where AI tools rely on large-scale aggregation of clinical and operational data.
- **Contractual complexity**, including indemnification and risk allocation between providers and AI developers.

HHS can play a constructive role by:

- Providing **guidance on governance and accountability structures** for non-device AI.
- Promoting **best practices for procurement, contracting, and risk management**.
- Ensuring privacy and security requirements are **clear, consistent, and technology-neutral**.

4. *For non-medical devices, what evaluation methods and metrics are most promising? Should HHS support these processes?*

Promising evaluation approaches for non-medical device AI include:

- **Pre-deployment validation using real-world clinical workflows**, not artificial test environments.
- **Post-deployment monitoring of performance drift**, user interaction, and workflow impact.
- **Human-centered evaluation**, assessing how clinicians interact with AI outputs and incorporate them into decision-making.

HHS could support these efforts through:

- **Targeted grants or cooperative agreements** focused on real-world evaluation methodologies.
 - **Public–private collaborations** to develop shared metrics and benchmarks.
 - **Pilot programs** that assess AI performance across diverse care settings.
5. *How can HHS best support private sector activities such as accreditation, certification, and industry-driven testing?*

HHS should support—but not replace—private sector efforts by:

- **Recognizing industry-led validation and accreditation programs** where appropriate.
- **Avoiding mandatory federal certification regimes** that could crowd out innovation or duplicate FDA oversight.
- Encouraging **voluntary, outcome-focused standards** that promote quality while allowing flexibility.

Private sector leadership has been critical to progress in radiology IT and AI, and similar models should be encouraged for broader clinical AI adoption.

6. *Where have AI tools met or fallen short of expectations? What tools show the greatest potential?*

AI tools have met or exceeded expectations in areas such as:

- **Imaging-based cancer screening**, where AI improves sensitivity, reduces false positives, and supports earlier diagnosis.
- **Workflow optimization**, including worklist prioritization and operational efficiency.

They have fallen short where:

- **Integration into clinical workflows is poor**, or
- **Payment and infrastructure barriers prevent consistent use.**

AI tools with the greatest future potential include those that:

- Support **early disease detection and risk stratification**,
- **Reduce unnecessary utilization**, and
- **Improve consistency and quality at scale.**

7. *Who influences AI adoption within health care organizations and what are the primary administrative hurdles?*

AI adoption is influenced by:

- Clinical leadership (e.g., radiologists and other physicians),
- Health system executives, and
- IT and compliance teams.

Key administrative hurdles include:

- Contracting and procurement complexity,
- Integration with existing IT systems,
- Unclear reimbursement and return-on-investment calculations, and
- Regulatory and compliance uncertainty.

Reducing these hurdles would materially accelerate adoption.

8. *Where would enhanced interoperability accelerate AI development and adoption?*

Enhanced interoperability would significantly advance AI adoption by enabling access to:

- **Longitudinal clinical outcomes data,**
- **Pathology and laboratory results** linked to imaging, and
- **Standardized metadata and annotations.**

Radiology's success with PACS and DICOM demonstrates how standardized data exchange can fuel innovation. Expanding similar interoperability across clinical domains would widen market opportunities and improve AI performance.

9. *What challenges do patients and caregivers want AI to address, and what concerns do they have?*

Patients and caregivers want AI to:

- Enable **earlier and more accurate diagnoses,**
- **Reduce unnecessary testing and delays,** and
- **Improve access to high-quality care.**

Concerns include:

- Data **privacy and security** and
- **Bias and fairness.**

Bias in AI is a legitimate concern, but it is not unique to AI—and it is best addressed through robust validation, diverse training data, and real-world performance monitoring. FDA already requires evidence that AI-enabled medical devices perform safely and effectively across diverse populations. In radiology, large-scale datasets drawn from varied geographic, racial, and socioeconomic populations are critical to building trustworthy algorithms

These concerns are best addressed through performance transparency, robust validation, and clinician oversight.

10. *Are there specific areas of AI research HHS should prioritize?*

HHS should prioritize research that:

- Evaluates **real-world clinical performance and outcomes**,
- Assesses **longitudinal impact on quality and costs**,
- Supports **post-market learning and continuous improvement**, and
- Preparing for **increasingly autonomous AI**.

Today, most AI tools support clinician decision-making. However, as evidence grows and trust increases, some AI applications—particularly in high-volume, low-risk screening contexts—may become partially or fully autonomous.

When AI is proven to perform as well as or better than human experts, policymakers should be open to revisiting existing regulatory and payment frameworks to ensure they do not become barriers to access, innovation, or efficiency.

(a) Published Findings

The literature increasingly demonstrates that AI can improve diagnostic accuracy and efficiency, particularly in imaging-based screening and triage, when appropriately validated and deployed. Studies have shown that general radiologists using AI technologies perform as well as or better than specialty-trained ones without AI, which raises the standard of care particularly in underserved communities.

(b) Costs, Benefits, and Transfers

Existing studies show that AI often shifts costs upfront but generates downstream savings through earlier detection, reduced utilization, and improved care coordination—benefits that current payment models do not fully capture.

In conclusion, AI-enabled imaging technologies are already delivering measurable benefits to patients and the healthcare system. AI-enabled technologies improve patient access to high quality care by



making the existing supply of radiologists and radiological technologists more efficient and effective. The earlier detection of disease improves outcomes and makes healthcare less expensive. To accelerate adoption responsibly, HHS should focus on:

- Regulatory clarity and inter-agency alignment,
- Payment policies that reward value and outcomes, and
- Data infrastructure that supports innovation while protecting privacy.

RadNet appreciates HHS' leadership in this area and stands ready to serve as a resource as policies are advanced that support safe, effective, and equitable AI in clinical care. If you have any questions or need additional information, please contact Michael Mabry, RadNet's Senior Director of Public Policy and Economic Analysis at 443.810.4798 or Michael.Mabry@RadNet.com.

Sincerely,

A handwritten signature in blue ink, appearing to read "Steve Forthuber".

Steve Forthuber
President and Chief Executive Officer
Eastern Operations

A handwritten signature in blue ink, appearing to read "Norman Hames".

Norman Hames
President and Chief Executive Officer
Western Operations